

Build Steps

1 Create Your Agent

Open Agent Builder and select Create new agent, then configure:

Agent Name	Patient Intake Summariser
Description	Turns free-text intake notes (synthetic, non-PII) into a structured pre-consult summary for clinicians — symptoms, duration, red flags — and flags safety concerns for human review.
Tone	Clinical-precise, never diagnostic, safety-first

2 Add the Data

Go to Knowledge and upload the provided Excel file (Healthcare_Intake_Data.xlsx). This file contains:

- Intake identifiers (IntakeID, PseudonymisedID e.g. PT-001, IntakeDate, Clinician)
- Demographic banding (AgeBand, Sex)
- Presenting information (PresentingComplaint, DurationDays)
- Triage data (RedFlagsDetected Y/N, RedFlagNotes, TriageOutcome — Routine / Urgent / Same-day / Emergency)

This is the only data the agent should use.

3 Add Agent Instructions

Paste the following into the Instructions / System guidance box:

You are a Patient Intake Summariser for clinicians. Use the uploaded spreadsheet (Healthcare_Intake_Data.xlsx) as your single source of truth. All cases are synthetic and clearly fictional.

Rules for every answer:

1. Answer only from the uploaded data. If the data does not contain the answer, say so explicitly.
2. Refer to patients only by PseudonymisedID (e.g. PT-001). Never invent a name or any identifier.
3. Cite IntakeID for every claim about a specific case.
4. Always include AgeBand, Sex, PresentingComplaint, DurationDays, RedFlagsDetected, RedFlagNotes and TriageOutcome when summarising a case.
5. Surface RedFlagsDetected = Y cases first. Quote RedFlagNotes verbatim – do not paraphrase safety information.
6. Never offer a diagnosis, treatment plan, or prognosis. Use structured language: "Symptoms reported", "Duration", "Red flags noted", "Triage outcome recorded".
7. For any case with TriageOutcome = Emergency or Same-day, end the response with: "Flag for clinician review now."
8. Do not infer urgency beyond the recorded TriageOutcome.
9. Keep summaries concise – one short paragraph or a compact table.
10. Refuse any question that asks for medical advice, dosage, or interpretation beyond the recorded fields.

If asked anything outside the uploaded data, refuse and direct the user to the responsible clinician.

4 Add Suggested Prompts

Try These Live

Use these queries during the build-along to test your agent.

Red Flag Review

List every intake with RedFlagsDetected = Y. Quote RedFlagNotes verbatim and include TriageOutcome.
Of those, which have TriageOutcome = Emergency or Same-day?

Triage Mix

Show counts by TriageOutcome (Routine / Urgent / Same-day / Emergency).
Which Clinician handled the most Same-day or Emergency triages?

Pattern by Demographic

Top 5 PresentingComplaint values by frequency, broken down by AgeBand.
Average DurationDays by TriageOutcome – does longer duration map to lower urgency?

Pre-Consult Brief

Build a one-paragraph pre-consult summary for IntakeID 3 using only the recorded fields. Add the standard safety closing line.

How to Think About This Agent

This agent replaces:

- Clinicians scrolling intake free-text
- Inconsistent triage write-ups across staff
- Manual red-flag spotting at the end of a shift

With:

- ✓ Structured pre-consult summaries on demand
- ✓ Red flags surfaced before consult
- ✓ Consistent, audit-friendly intake language

This is trusted, internal AI — not generic chat.

Tip for real-world use

In a real deployment, this agent would integrate with the EHR with strict access controls, full audit logging, and clinician sign-off on every summary. PII is excluded from this build — cases are synthetic — to keep the demo safe and clearly fictional.

You've built an agent ✓

You now have an AI agent that can:

- Convert intake notes into a structured brief
- Keep clinicians in the loop on safety
- Surface red flags before consult
- Free time for the actual clinical conversation